



Fiona Stanley Fremantle Hospitals Group

Policy and Procedure

Duress response and management

Reference #:FSFH-MENH-PPOL-0001

Scope

Site	Service/Department/Unit	Disciplines
Fiona Stanley Hospital	Cockburn Health	Medical, Nursing, Allied Health, Safety and Incident Management Services (SIMS) and Facilities Management

1. Introduction

The Mental Health Service (MHS) is required to meet its workplace health and safety responsibilities to provide a safe working environment for staff and protect them from harm. The public also expect their healthcare will be delivered in a safe environment.

Whilst duress alarms do not prevent an incident occurring, they do enable staff to call for immediate assistance when in a duress situation. The activation of a duress alarm at Cockburn Health automatically produces a Code Black response. This document outlines the requirements and processes for use of individual duress alarms by staff at Cockburn Health.

2. Terminology

Advance Care IP RTLS Personal Duress Pendant	A personal alarm device carried by an individual staff member.
CHoIR	Combined Hazard or Incident Report. System used to report hazards or incidents.
Code Black	Emergency procedure where staff members who feel that their personal safety, or the safety of others, is under threat and can summon a prompt response from appropriately trained personnel.
Datix CIMS	Clinical Incident Management System. Used for the tracking

	and notifying of clinical incidents.
DMR	Digital Medical Record
RTLS	Real Time Location System provided by Advance Care IP™ to address communication, workflow, patient safety within Nurse call systems.
Staff assist	Request for staff assistance in a non-code situation.

3. Policy

- All Cockburn Health staff will be orientated to the local Emergency procedure codes on commencement of employment.
- All staff must always wear an Advance Care IP™ RTLS personal duress pendant when on duty.
- Staff must be provided with training on the use of the Advance Care IP™ RTLS personal duress pendant.
- All staff will ensure their personal pendant is in working order on each shift.
- Only designated response team members will respond directly to the location of the emergency.
- Debriefing, support, and review of clinical practices will be provided for staff as soon as possible following any incident involving a duress activation.
- All duress pendants remain the property of Cockburn Health and must be returned at termination of employment.
- Personal duress pendants issued for temporary use must be returned at the end of the shift.

4. Procedure

4.1. Staff training

- All clinical and non-clinical staff including casual or visiting staff will be provided with a duress pendant by the ward NUM or HOOT during orientation and informed of the following:
 - location of fixed and wall mounted duress alarms.
 - functioning and testing of duress pendants.
 - when to call a staff assist or an emergency duress for any emergency code.
 - their role in responding to a duress activation.
 - how to cancel a duress alarm.

4.2. Advance Care IP™ RTLS Personal duress pendant

- The Advance Care™ RTLS personal duress pendant (see Figure 1) has a control panel which has a **RED** call button, a **GREY** call button and LED light display between the two buttons which will flash in various colours depending on the function progress.



Figure1: Advance Care IP™ RTLS Personal duress pendant

- On the bottom of the RTLS is a micro-USB charging port.
- Within the device is an audible alarm which will beep at various rates depending on the function in progress.
- Staff must wear their duress pendant, so it remains uncovered and easily accessible. Staff must not carry in a pocket.
- Staff must keep all liquids, including hand sanitizers away from the RTLS or it may cease to function correctly.
- Staff will escalate via their line manager if unable to locate their personal duress pendant at the commencement of their shift.

4.3. Charging the RTLS personal duress pendant

- 4.3.1. Locate the USB port on the bottom of the device.
- 4.3.2. Plug the micro-USB cable into the port and plug the other end of the cable into a charging port.
- 4.3.3. The personal duress pendant will beep twice, and the LED light will flash **green** before charging and then **amber** indicating the pendant is charging.

4.3.4. Once fully charged the LED light will change to **green**.

4.3.5. Unplug the USB cable.

4.3.6. The duress pendant can now be used.

4.4. Testing

- Personal duress pendants must be tested prior to commencing each shift to ensure the battery is fully functional.
- Staff will test the personal duress pendant by opening the Advanced Care™ system application on a computer and confirming that the system has picked up their device noted by a blinking signal.

4.5. Activating the RTLS personal duress pendant

- Duress calls are made by pressing the buttons on the control panel of the pendant (Figure 2).

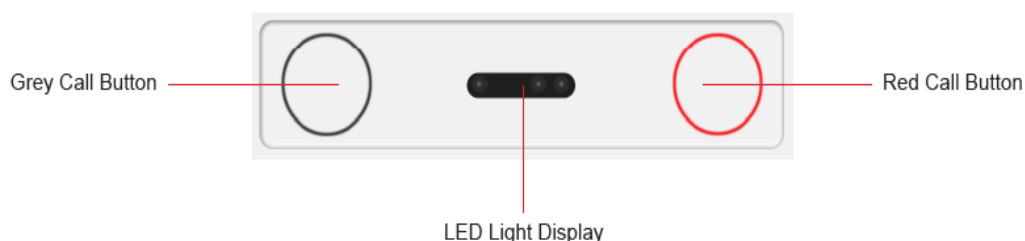


Figure 2: Personal duress pendant control panel

There are two modes to call a duress:

- Staff assist.
 - Press the grey call button first and then while holding down the **grey** call button, press the **red** call button.
 - The duress pendant will emit two quick beeps, then become silent. The LED light will flash **green**.
 - The light will continue to flash rapidly until the call is cancelled.
 - There is no need to cancel a staff assist to change to a duress response.
 - If a staff assist is in progress, press the **grey** and **red** call button together as described above to escalate to a duress response.
- Code Black duress.
 - Press the **red** call button once.
 - The LED light flashes **green** and then pendant beeps rapidly.
 - The light will flash and the pendant will beep intermittently until the alarm is cancelled

When a staff member activates their RTLS Personal Duress Pendant they must remain at the location of the duress (if safe to do so) to allow the location to be clearly identified.

4.6. Duress alarm location

- When a personal duress pendant is activated an audible alert alarm will sound
- The unit location is indicated on:
 - annunciator notification (located at each nurse's station).
 - map of the staff members location will be displayed on a computer screen on the unit to notify staff of duress location.

4.7. Emergency response team

- In the event a duress is activated, the Emergency Response Team (ERT) will automatically respond and immediately attend the location of the emergency.
 - Staff are to be guided by the Action Cards listed in the appendix of the Cockburn Health Emergency Codes policy.
 - Staff responding to a personal duress alarm will not put themselves at risk and will wait until adequate staff are present to assess and manage the situation.
- 4.7.1. Establish the nature of the situation and coordinate the appropriate response, directing other staff as necessary.
- 4.7.2. Determine the need to remove other patients from the area.
- 4.7.3. In discussion with SIMS determine if a police response is required. If so, request a staff member to call '000' for urgent police assistance. State the name of the service, reason for the call and location.
- 4.7.4. Other staff will not respond directly to the emergency unless otherwise requested to do so.
- 4.7.5. All other clinical and non-clinical staff will provide back up support for an emergency duress situation and support the supervision for patients and visitors, not involved in the incident.
- 4.7.6. Escalate to medical and the multidisciplinary team if risks are increased either during or after the code. Any changes to care will be clearly handed over, using the iSoBAR format.
- 4.7.7. If the situation cannot be resolved, and aggression continues to escalate, consider contacting WA Police for assistance.

4.8. Cancelling a duress.

- 4.8.1. When the incident has been stood down, or the person that raised the initial alarm has confirmed a false alarm, the duress will need to be cancelled.
- 4.8.2. Press the **grey** call button once on the personal duress pendant that was activated.
- 4.8.3. The LED light stops flashing and the pendant stops beeping.
- 4.8.4. The pendant is now reset and normal use can continue.

4.9. Documentation

- All code activations are documented by the CNS/NUM/HOOT CNM in the 24-hour report.
- Following the occurrence of an emergency duress, staff involved will:
 - document in the patient's medical record (DMR)
 - complete a Datix Clinical Incident Monitoring System (CIMS) notification.
 - complete a Combined Hazard or incident report (CHoIR) form.
 - alert on PSOLIS if clinically indicated.
- Staff will be provided with an opportunity to debrief with their supervisor or manager on duty and encouraged to access the Employee Assistance Program or FSFHG Staff Psychological Wellbeing and Support (PWS) Program if required.

4.10. Maintenance

- Escalate any issues with the system to the senior nurse on duty.
- If a duress pendant is lost or becomes damaged, contact SIMS on 6171 4950 to generate a work order for SIMS to replace the pendant.
- Fixed and wall mounted duress alarms must be tested every three months.

4.11. Duress system downtime procedure

- Senior nurse on duty to review the Cockburn Health Business Continuity Plan and determine whether to request to activate Code Yellow via the HIC and escalate and implement work arounds as appropriate.
- Shift coordinators to organise for staff on their ward to work in pairs to ensure safe working environment until the duress system is functioning properly.

5. Compliance/performance monitoring

- The Nurse Unit Manager will monitor behaviour incidents reported in DATIX CIMS and trended clinical incident reports are monitored monthly at the Service Safety Quality & Risk Committee.

- The Service Performance and Corporate Leadership Committee will review all Code Black data monthly to identify trends and new high-risk areas.
- Compliance with the API training is monitored by the Performance and Corporate Leadership Committee monthly.
- Site duress alarm system testing will be audited annually, with results submitted to the FSFHG Emergency Preparedness Disaster Management Committee.

6. Related policy documents

- SMHS
 - [Duress alarm systems](#)
 - [Work health and safety](#)
 - [Workplace aggression and violence policy](#)
 - [Workplace aggression and violence procedure](#)
 - [Critical Incident stress management](#)
 - [Employee assistance program](#)
- FSFHG
 - Recognising & Responding to Acute Deterioration (FSFH-HW-POL-0033)
- Office of the Chief Psychiatrist of Western Australia
 - [Policies for Mandatory Reporting of Notifiable Incidents to the Chief Psychiatrist](#)

7. Other related resources

- Code Black – Cockburn Health
- Cockburn Health Emergency Codes

8. Related standards

- NSQHS Standards
 - Clinical Governance
 - Comprehensive Care
 - Communicating for Safety
 - Recognising and Responding to Acute Deterioration

- [Chief Psychiatrists Standards for Clinical Care](#)
 - Risk assessment and management

9. Authorisation

EXECUTIVE SPONSOR: Nurse Director, Service 5					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Endorsed By	Revision due
0.9	09/2024	Quality Safety and Risk Officer, Cockburn Health	Service 5 SQR	Service 5 SQR Committee	09/2027